

PMC Compass: Modelling Outputs for TGO

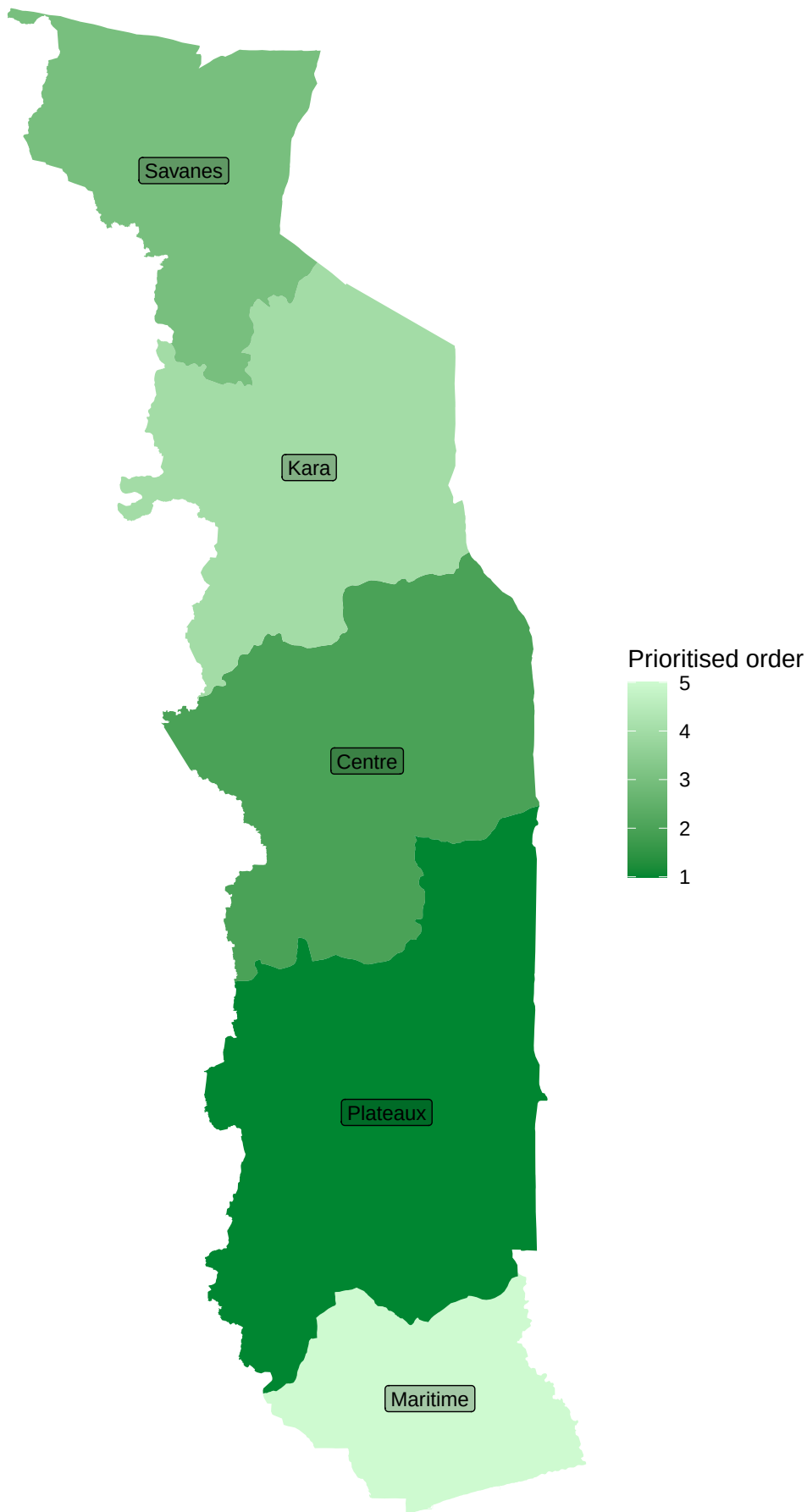
Modelling outputs from PMC Compass. Please note, results are preliminary and subject to change following further validation.

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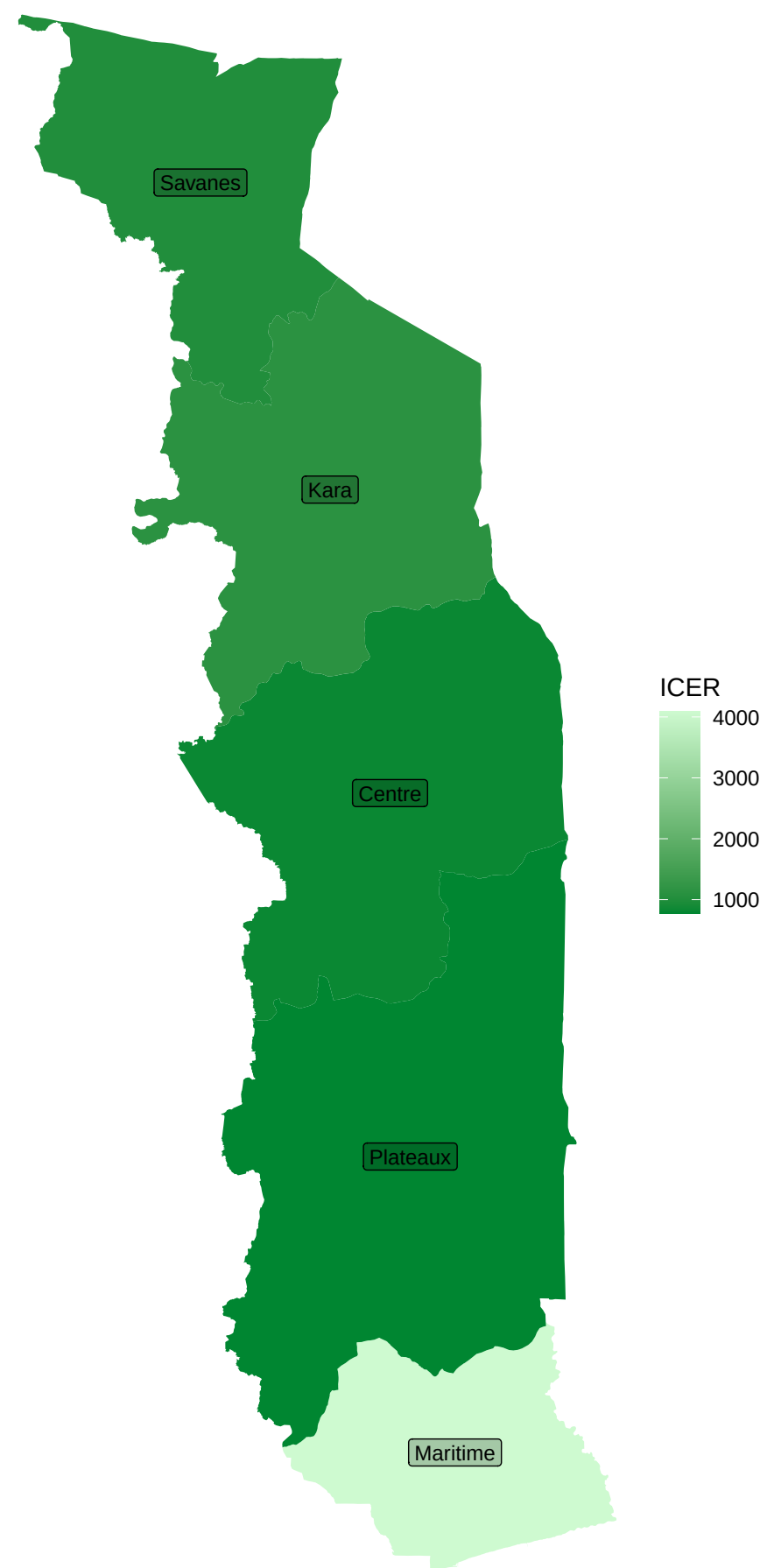
TGO: Ranked prioritised order to implement PMC

Order at which to implement PMC in the country based on cost-effectiveness of the PMC delivery strategy.



TGO: Incremental cost-effectiveness ratios of implementing PMC co-delivery with EPI only vs No PMC in each admin-1 unit.

Incremental cost-effectiveness ratios of implementing PMC co-delivery with EPI only vs No PMC in each admin-1 unit. All costs used to calculate these exclude national-level start up costs.



TGO: Ranked prioritisation of options (page 1)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Additional clinical malaria cases averted	Additional hospitalisations averted	Additional deaths averted	Additional DALYs averted
TGO	Plateaux	PMC co-delivery with EPI only vs No PMC	6179	311	67	424
TGO	Centre	PMC co-delivery with EPI only vs No PMC	2392	133	29	169
TGO	Savanes	PMC co-delivery with EPI only vs No PMC	2638	172	37	195
TGO	Kara	PMC co-delivery with EPI only vs No PMC	2429	145	31	174
TGO	Maritime	PMC co-delivery with EPI only vs No PMC	1013	67	14	74

TGO: Ranked prioritisation of options (page 2)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Financial cost of SP and administration consumables	Cumulative cost of SP and administration consumables	Additional economic cost savings to public providers from reduced treatment	Cumulative economic cost savings to public providers from reduced treatment
TGO	Plateaux	PMC co-delivery with EPI only vs No PMC	28608	28608	-31109	-31109
TGO	Centre	PMC co-delivery with EPI only vs No PMC	15720	44328	-12812	-43921
TGO	Savanes	PMC co-delivery with EPI only vs No PMC	21960	66288	-15674	-59595
TGO	Kara	PMC co-delivery with EPI only vs No PMC	16550	82838	-13615	-73210
TGO	Maritime	PMC co-delivery with EPI only vs No PMC	69392	152230	-6079	-79289

TGO: Ranked prioritisation of options (page 3)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Additional net economic costs of PMC implementation and treatment	Cumulative net costs of PMC implementation and treatment	Incremental economic cost per DALY averted vs. next higher option in ranking	Cost-effective? (lower country-specific threshold)
TGO	Plateaux	PMC co-delivery with EPI only vs No PMC	478065	478065	1128	no
TGO	Centre	PMC co-delivery with EPI only vs No PMC	140555	618620	832	no
TGO	Savanes	PMC co-delivery with EPI only vs No PMC	199187	817807	1021	no
TGO	Kara	PMC co-delivery with EPI only vs No PMC	197077	1014884	1133	no
TGO	Maritime	PMC co-delivery with EPI only vs No PMC	302196	1317080	4084	no

TGO: Ranked prioritisation of options (page 4)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Cost-effective? (higher country-specific threshold)	Cost-effective? (\$250 / DALY averted threshold)	Population (all ages)	Expected PMC doses delivered (age 0-1yr)
TGO	Plateaux	PMC co-delivery with EPI only vs No PMC	no	no	1846054	178797
TGO	Centre	PMC co-delivery with EPI only vs No PMC	no	no	881285	98249
TGO	Savanes	PMC co-delivery with EPI only vs No PMC	no	no	1228375	137247
TGO	Kara	PMC co-delivery with EPI only vs No PMC	no	no	985935	103435
TGO	Maritime	PMC co-delivery with EPI only vs No PMC	no	no	4214922	433702

TGO: Ranked prioritisation of options (page 5)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Expected PMC doses delivered (age 1-2yr)	Expected PMC doses delivered (age 2-2.5yr)	Data source for cost modelling	ICER based on regional costs only (no national costs)
TGO	Plateaux	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	783
TGO	Centre	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	832
TGO	Savanes	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	1021
TGO	Kara	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	1133
TGO	Maritime	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	4084

TGO: Ranked prioritisation of options (page 6)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	rank
TGO	Plateaux	PMC co-delivery with EPI only vs No PMC	1
TGO	Centre	PMC co-delivery with EPI only vs No PMC	2
TGO	Savanes	PMC co-delivery with EPI only vs No PMC	3
TGO	Kara	PMC co-delivery with EPI only vs No PMC	4
TGO	Maritime	PMC co-delivery with EPI only vs No PMC	5

TGO: Cost assumptions (page 1)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	N doses in PMC schedule	Data source for cost modelling	Total cost of PMC implementation (excl national costs)
TGO	Centre	881285	No PMC	0	Plus Project countries	0
TGO	Centre	881285	PMC co–delivery with EPI only	4	Plus Project countries	153367
TGO	Kara	985935	No PMC	0	Plus Project countries	0
TGO	Kara	985935	PMC co–delivery with EPI only	4	Plus Project countries	210692
TGO	Maritime	4214922	No PMC	0	Plus Project countries	0
TGO	Maritime	4214922	PMC co–delivery with EPI only	4	Plus Project countries	308275
TGO	Plateaux	1846054	No PMC	0	Plus Project countries	0
TGO	Plateaux	1846054	PMC co–delivery with EPI only	4	Plus Project countries	363117
TGO	Savanes	1228375	No PMC	0	Plus Project countries	0
TGO	Savanes	1228375	PMC co–delivery with EPI only	4	Plus Project countries	214861

TGO: Cost assumptions (page 2)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Sub–total: Cost of treatment	Total cost of PMC implementation + treatment (excluding national costs)	Total cost of PMC implementation + treatment (including full national costs for every region)
TGO	Centre	881285	No PMC	205192	205192	205192
TGO	Centre	881285	PMC co–delivery with EPI only	192380	345747	491804
TGO	Kara	985935	No PMC	212244	212244	212244
TGO	Kara	985935	PMC co–delivery with EPI only	198629	409321	555378
TGO	Maritime	4214922	No PMC	113329	113329	113329
TGO	Maritime	4214922	PMC co–delivery with EPI only	107250	415525	561582
TGO	Plateaux	1846054	No PMC	527510	527510	527510
TGO	Plateaux	1846054	PMC co–delivery with EPI only	496401	859518	1005575
TGO	Savanes	1228375	No PMC	243972	243972	243972
TGO	Savanes	1228375	PMC co–delivery with EPI only	228298	443159	589216

TGO: Cost assumptions (page 3)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Clinical cases (N, 0–30mo)	Clinical cases for whom care sought	Hospitalisations (N, 0–30mo)
TGO	Centre	881285	No PMC	42221	21110	2004
TGO	Centre	881285	PMC co–delivery with EPI only	39829	19914	1871
TGO	Kara	985935	No PMC	43761	21880	2070
TGO	Kara	985935	PMC co–delivery with EPI only	41332	20666	1925
TGO	Maritime	4214922	No PMC	20239	10120	1206
TGO	Maritime	4214922	PMC co–delivery with EPI only	19226	9613	1139
TGO	Plateaux	1846054	No PMC	131041	65520	4427
TGO	Plateaux	1846054	PMC co–delivery with EPI only	124862	62431	4116
TGO	Savanes	1228375	No PMC	47584	23792	2467
TGO	Savanes	1228375	PMC co–delivery with EPI only	44946	22473	2295

TGO: Cost assumptions (page 4)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Deaths (N, 0–30mo)	Total DALYs	DALYs from clinical cases
TGO	Centre	881285	No PMC	431	2851	2153
TGO	Centre	881285	PMC co–delivery with EPI only	402	2682	2031
TGO	Kara	985935	No PMC	445	2952	2232
TGO	Kara	985935	PMC co–delivery with EPI only	414	2778	2108
TGO	Maritime	4214922	No PMC	259	1451	1032
TGO	Maritime	4214922	PMC co–delivery with EPI only	245	1377	981
TGO	Plateaux	1846054	No PMC	952	8224	6683
TGO	Plateaux	1846054	PMC co–delivery with EPI only	885	7800	6368
TGO	Savanes	1228375	No PMC	530	3285	2427
TGO	Savanes	1228375	PMC co–delivery with EPI only	493	3090	2292

TGO: Cost assumptions (page 5)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	DALYs from hospitalisations	DALYs from deaths	PMC cost input: Implementation cost at national level
TGO	Centre	881285	No PMC	267	431	0
TGO	Centre	881285	PMC co–delivery with EPI only	249	402	146057
TGO	Kara	985935	No PMC	275	445	0
TGO	Kara	985935	PMC co–delivery with EPI only	256	414	146057
TGO	Maritime	4214922	No PMC	160	259	0
TGO	Maritime	4214922	PMC co–delivery with EPI only	151	245	146057
TGO	Plateaux	1846054	No PMC	589	952	0
TGO	Plateaux	1846054	PMC co–delivery with EPI only	547	885	146057
TGO	Savanes	1228375	No PMC	328	530	0
TGO	Savanes	1228375	PMC co–delivery with EPI only	305	493	146057

TGO: Cost assumptions (page 6)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	PMC cost input: PMC implementation cost per district	PMC cost input: PMC implementation cost per health facility	PMC cost input: Cost of PMC administration consumables per dose
TGO	Centre	881285	No PMC	0	0	0.00
TGO	Centre	881285	PMC co–delivery with EPI only	27477	225	0.05
TGO	Kara	985935	No PMC	0	0	0.00
TGO	Kara	985935	PMC co–delivery with EPI only	27477	225	0.05
TGO	Maritime	4214922	No PMC	0	0	0.00
TGO	Maritime	4214922	PMC co–delivery with EPI only	27477	225	0.05
TGO	Plateaux	1846054	No PMC	0	0	0.00
TGO	Plateaux	1846054	PMC co–delivery with EPI only	27477	225	0.05
TGO	Savanes	1228375	No PMC	0	0	0.00
TGO	Savanes	1228375	PMC co–delivery with EPI only	27477	225	0.05

TGO: Cost assumptions (page 7)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	PMC Cost input: Cost per SP tablet (includes wastage)	N districts in region	N health facilities in region
TGO	Centre	881285	No PMC	0.00	5	23
TGO	Centre	881285	PMC co–delivery with EPI only	0.11	5	23
TGO	Kara	985935	No PMC	0.00	7	31
TGO	Kara	985935	PMC co–delivery with EPI only	0.11	7	31
TGO	Maritime	4214922	No PMC	0.00	9	59
TGO	Maritime	4214922	PMC co–delivery with EPI only	0.11	9	59
TGO	Plateaux	1846054	No PMC	0.00	12	61
TGO	Plateaux	1846054	PMC co–delivery with EPI only	0.11	12	61
TGO	Savanes	1228375	No PMC	0.00	7	33
TGO	Savanes	1228375	PMC co–delivery with EPI only	0.11	7	33

TGO: Cost assumptions (page 8)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	N expected PMC doses delivered (age 0–1yr)	N expected PMC doses delivered (age 1–2yr)	N expected PMC doses delivered (age 2–2.5yr)
TGO	Centre	881285	No PMC	0	0	0
TGO	Centre	881285	PMC co–delivery with EPI only	98249	0	0
TGO	Kara	985935	No PMC	0	0	0
TGO	Kara	985935	PMC co–delivery with EPI only	103435	0	0
TGO	Maritime	4214922	No PMC	0	0	0
TGO	Maritime	4214922	PMC co–delivery with EPI only	433702	0	0
TGO	Plateaux	1846054	No PMC	0	0	0
TGO	Plateaux	1846054	PMC co–delivery with EPI only	178797	0	0
TGO	Savanes	1228375	No PMC	0	0	0
TGO	Savanes	1228375	PMC co–delivery with EPI only	137247	0	0

TGO: Cost assumptions (page 9)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	N total expected PMC doses delivered (age 0–2.5yr)	N SP tablets delivered	Sub–total: Cost of PMC implementation at district and HFs
TGO	Centre	881285	No PMC	0	0	0
TGO	Centre	881285	PMC co–delivery with EPI only	98249	98249	142560
TGO	Kara	985935	No PMC	0	0	0
TGO	Kara	985935	PMC co–delivery with EPI only	103435	103435	199314
TGO	Maritime	4214922	No PMC	0	0	0
TGO	Maritime	4214922	PMC co–delivery with EPI only	433702	433702	260568
TGO	Plateaux	1846054	No PMC	0	0	0
TGO	Plateaux	1846054	PMC co–delivery with EPI only	178797	178797	343449
TGO	Savanes	1228375	No PMC	0	0	0
TGO	Savanes	1228375	PMC co–delivery with EPI only	137247	137247	199764

TGO: Cost assumptions (page 10)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Sub–total: Cost of PMC administration (SP+consumables)	Treatment cost per clinical case	Treatment cost per hospitalisation
TGO	Centre	881285	No PMC	0	3.93	50.2
TGO	Centre	881285	PMC co–delivery with EPI only	15720	3.93	50.2
TGO	Kara	985935	No PMC	0	3.93	50.2
TGO	Kara	985935	PMC co–delivery with EPI only	16550	3.93	50.2
TGO	Maritime	4214922	No PMC	0	3.93	50.2
TGO	Maritime	4214922	PMC co–delivery with EPI only	69392	3.93	50.2
TGO	Plateaux	1846054	No PMC	0	3.93	50.2
TGO	Plateaux	1846054	PMC co–delivery with EPI only	28608	3.93	50.2
TGO	Savanes	1228375	No PMC	0	3.93	50.2
TGO	Savanes	1228375	PMC co–delivery with EPI only	21960	3.93	50.2

TGO: Cost assumptions (page 11)

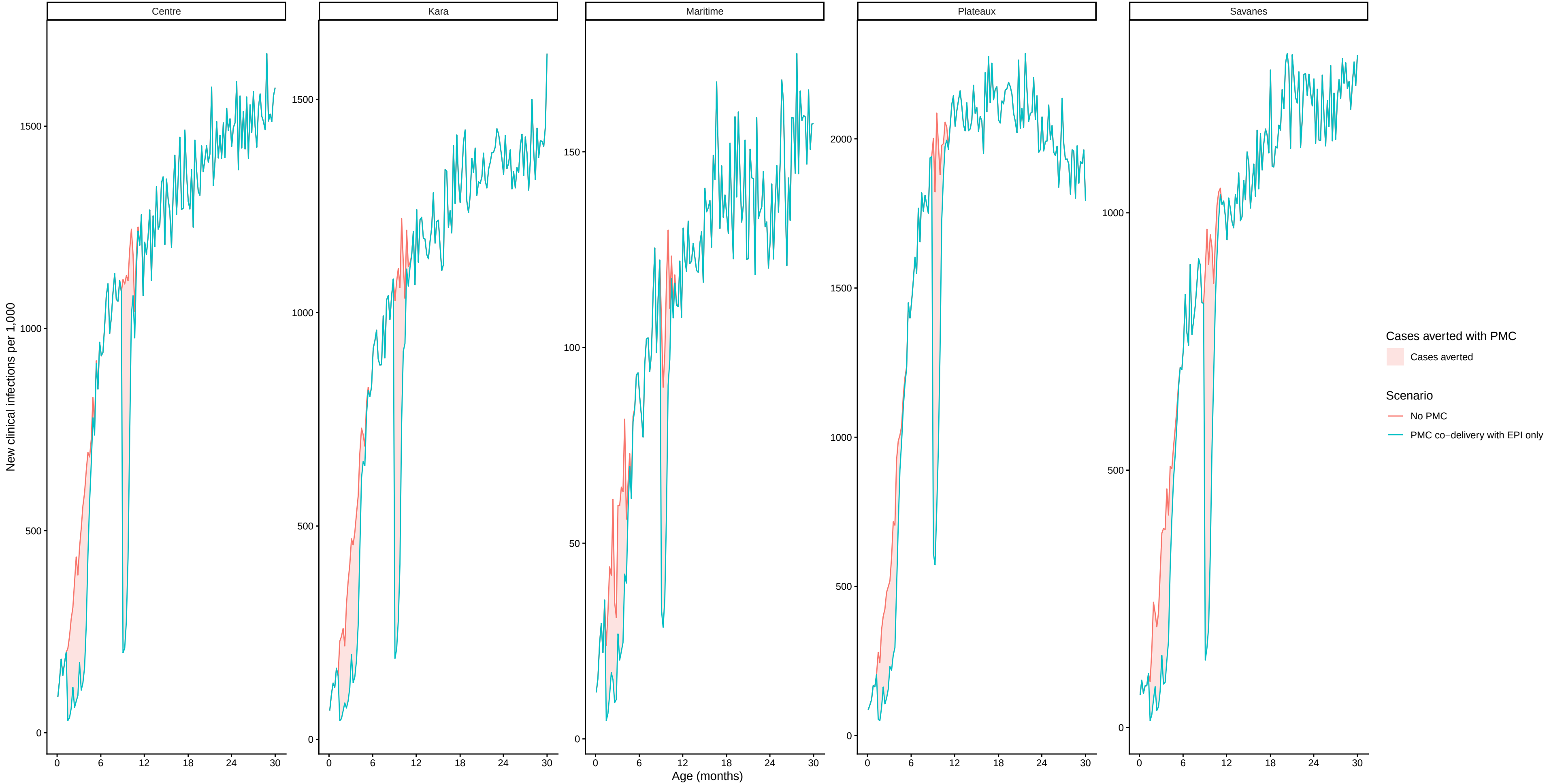
Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Treatment cost per death
TGO	Centre	881285	No PMC	50.2
TGO	Centre	881285	PMC co–delivery with EPI only	50.2
TGO	Kara	985935	No PMC	50.2
TGO	Kara	985935	PMC co–delivery with EPI only	50.2
TGO	Maritime	4214922	No PMC	50.2
TGO	Maritime	4214922	PMC co–delivery with EPI only	50.2
TGO	Plateaux	1846054	No PMC	50.2
TGO	Plateaux	1846054	PMC co–delivery with EPI only	50.2
TGO	Savanes	1228375	No PMC	50.2
TGO	Savanes	1228375	PMC co–delivery with EPI only	50.2

TGO: Clinical malaria incidence

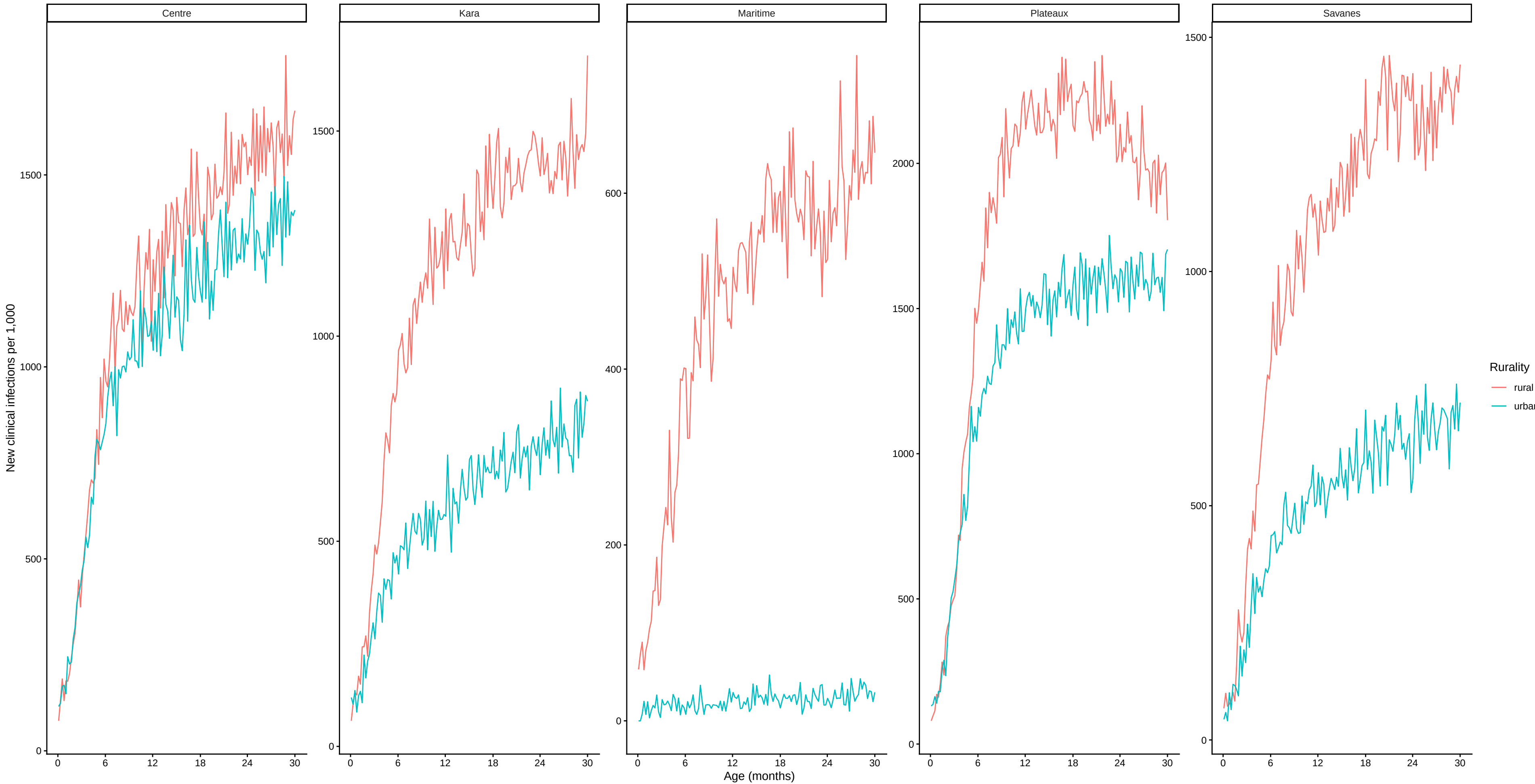
Age-specific clinical malaria cases per 1,000 children (0–30 months, with and without PMC).

PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.



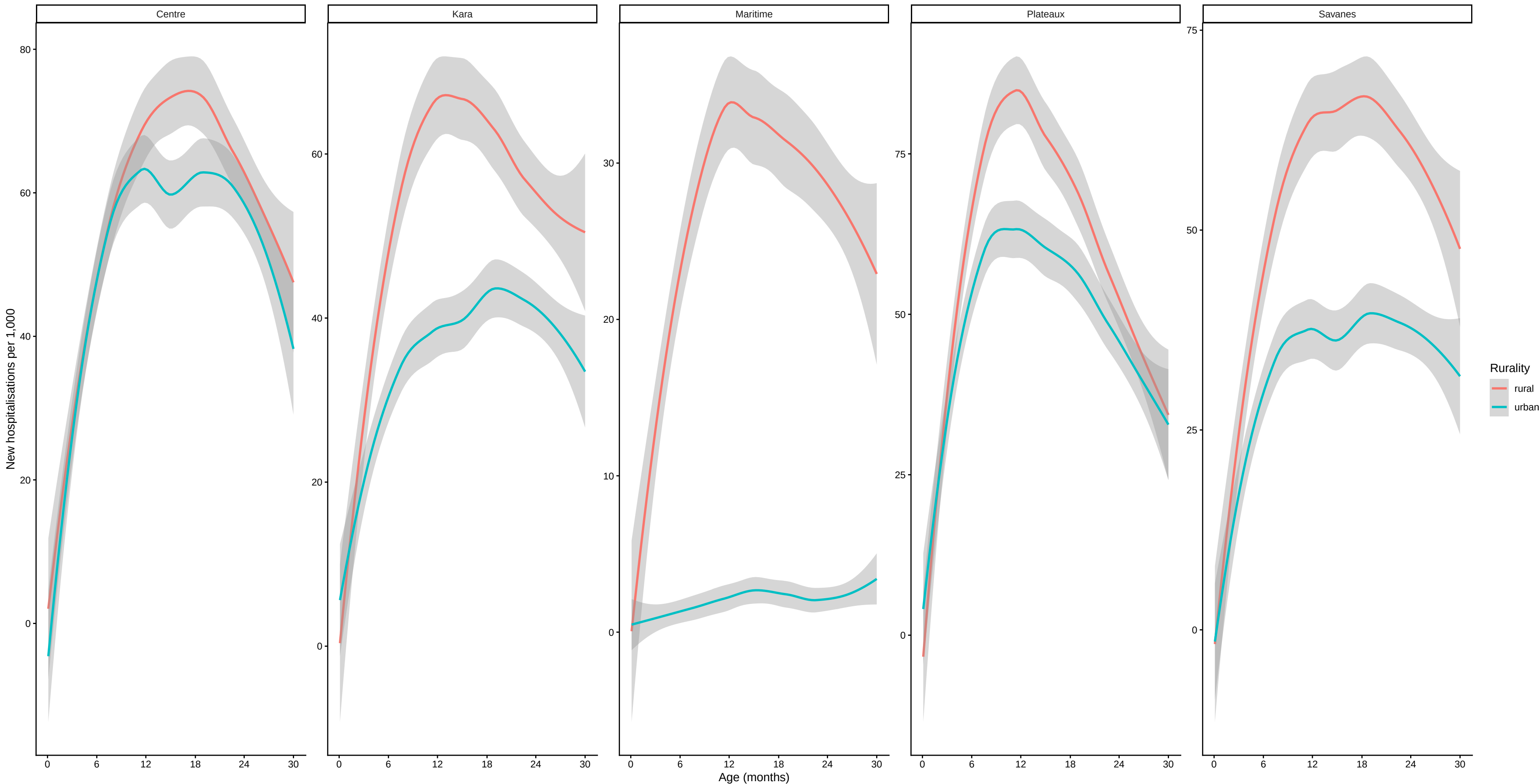
TGO: Clinical malaria incidence (rural and urban)

Age-specific clinical malaria cases per 1,000 children (0–30 months, no PMC), stratified by rural and urban residence.



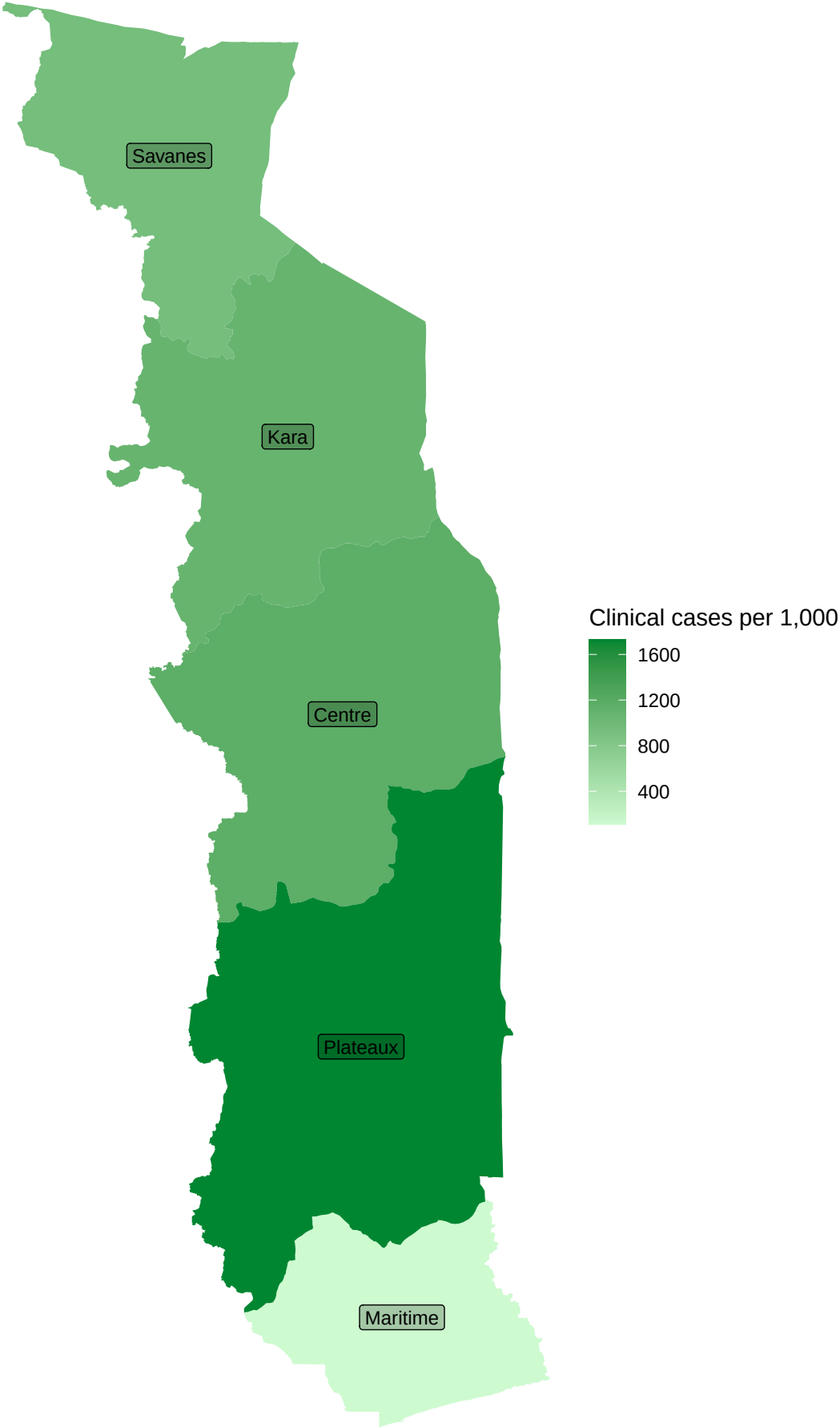
TGO: Malaria hospitalisations

Age-specific new malaria hospitalisations per 1,000 children (0–30 months, no PMC), stratified by rural and urban residence.
Raw model outputs have been smoothed to reduce stochastic variation.



TGO: Spatial distribution of clinical malaria without PMC

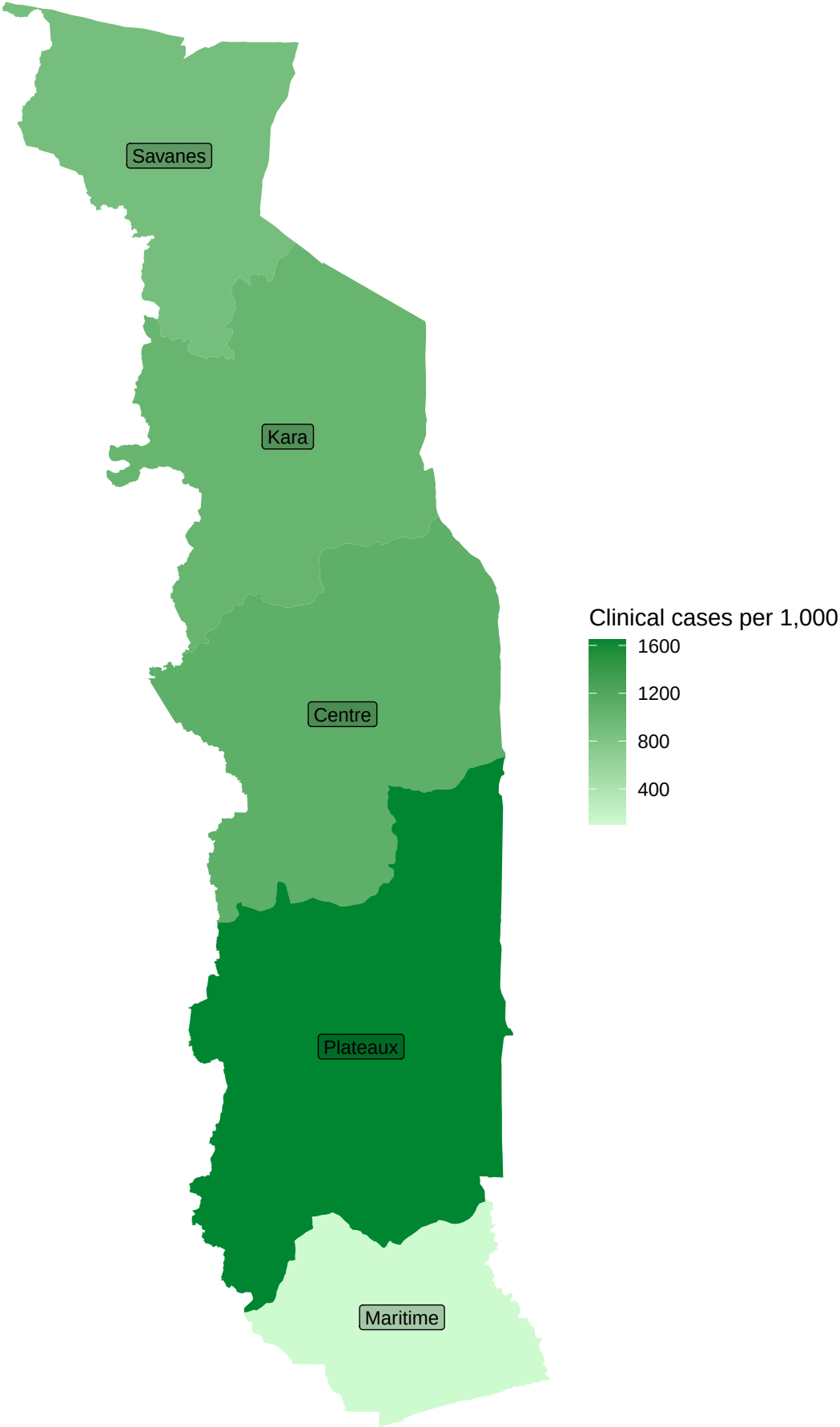
Admin-1 clinical malaria incidence per 1,000 children aged (0–30 months, no PMC).



TGO: Spatial distribution of clinical malaria with PMC

Admin-1 clinical malaria incidence per 1,000 children aged (0–30 months, with PMC).

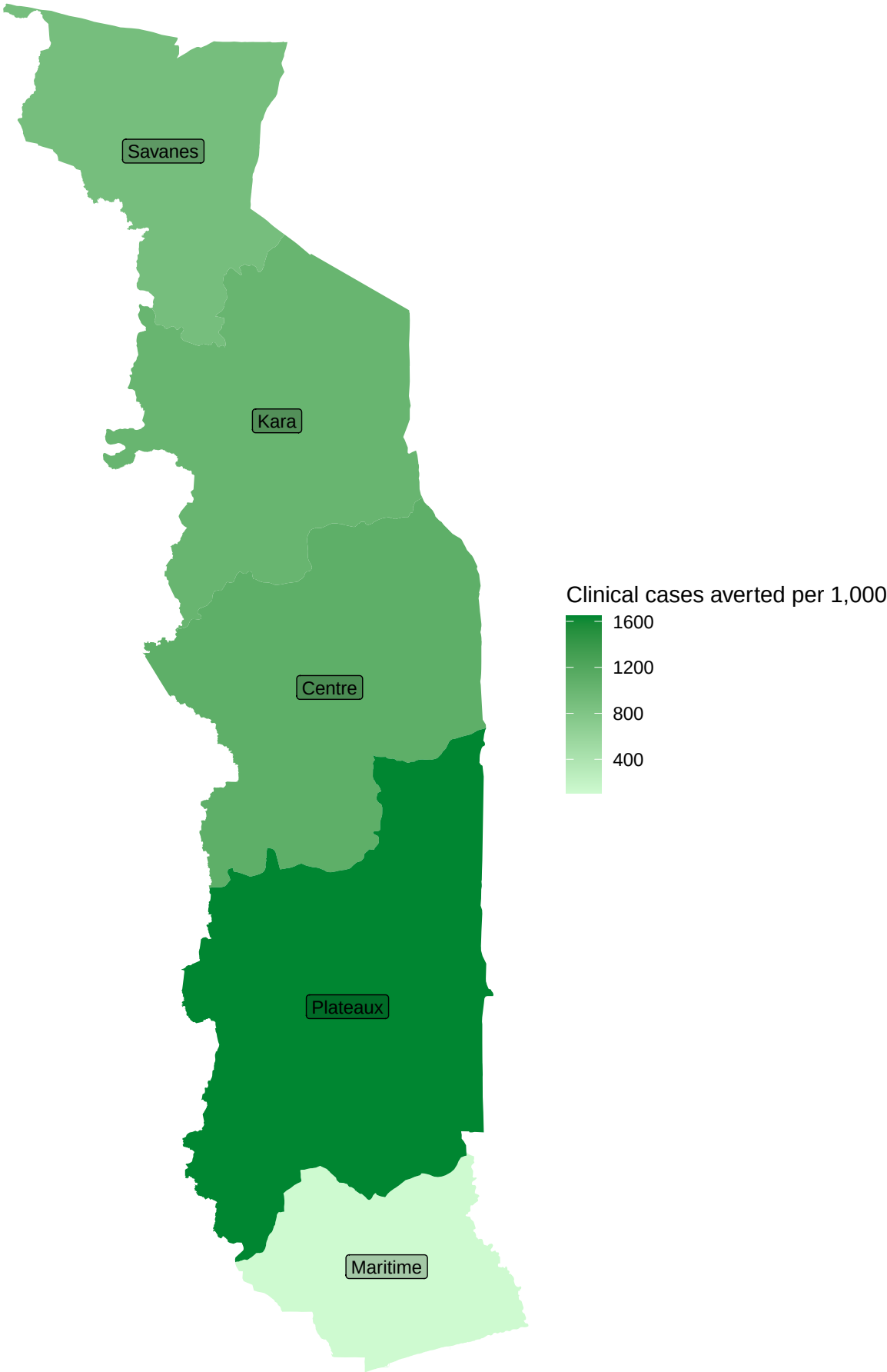
PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.



TGO: Spatial distribution of averted clinical malaria with PMC

Admin-1 averted clinical malaria incidence per 1,000 children aged (0–30 months, averted with PMC).

PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.



TGO: Clinical malaria burden (page 1)

Annual clinical malaria burden per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co–delivered with EPI and have a 90% uptake relative to the coverage of the co–delivered intervention.

Admin–1	Clinical cases per 1,000, no PMC (0–2.5y)	Clinical cases per 1,000, with PMC (0–2.5y)	Clinical cases averted per 1,000 (0–2.5y)
Abia	362.1	325.8	36.3
Adamawa	379.8	355.7	24.1
Akwa_Ibom	445.1	413.0	32.1
Anambra	247.1	225.2	21.9
Bauchi	474.3	457.9	16.4
Bayelsa	578.6	539.8	38.8
Benue	369.2	343.5	25.8
Borno	1287.6	1233.4	54.3
Cross_River	527.1	486.7	40.4
Delta	333.0	308.1	24.8
Ebonyi	752.8	694.8	58.1
Edo	705.8	650.6	55.2
Ekiti	1165.1	1065.5	99.7
Enugu	380.5	348.1	32.4
Federal_Capital_Territory	323.3	297.5	25.7
Gombe	448.2	431.8	16.4
Imo	260.2	239.5	20.7
Jigawa	1614.4	1517.4	96.9
Kaduna	594.6	570.2	24.4
Kano	826.2	788.8	37.3
Katsina	2178.7	2115.4	63.2
Kebbi	3879.9	3744.2	135.7
Kogi	606.9	568.5	38.5
Kwara	372.3	348.6	23.7
Lagos	123.6	111.7	11.9
Nasarawa	1022.6	952.2	70.4
Niger	1011.6	970.3	41.3
Ogun	412.8	389.4	23.4
Ondo	1074.9	992.2	82.7
Osun	718.5	679.2	39.3

TGO: Clinical malaria burden (page 2)

Annual clinical malaria burden per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co–delivered with EPI and have a 90% uptake relative to the coverage of the co–delivered intervention.

Admin–1	Clinical cases per 1,000, no PMC (0–2.5y)	Clinical cases per 1,000, with PMC (0–2.5y)	Clinical cases averted per 1,000 (0–2.5y)
Oyo	582.4	552.3	30.1
Plateau	786.5	733.0	53.5
Rivers	384.2	352.8	31.4
Sokoto	3696.7	3601.7	95.0
Taraba	563.2	540.4	22.8
Yobe	780.6	737.7	42.9
Zamfara	3789.7	3718.0	71.7

TGO: Severe malaria burden (page 1)

Annual severe malaria hospitalisations per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.

Admin-1	Hospitalisations per 1,000, no PMC (0–2.5y)	Hospitalisations per 1,000, with PMC (0–2.5y)	Hospitalisations averted per 1,000 (0–2.5y)
Abia	24.1	21.3	2.8
Adamawa	24.7	23.0	1.8
Akwa_Ibom	26.5	24.3	2.2
Anambra	16.9	15.3	1.7
Bauchi	30.9	29.7	1.2
Bayelsa	32.2	29.7	2.5
Benue	23.2	21.5	1.7
Borno	59.7	56.8	2.9
Cross_River	29.5	26.8	2.7
Delta	21.8	19.9	1.8
Ebonyi	38.3	34.9	3.4
Edo	36.8	33.1	3.7
Ekiti	48.1	42.8	5.4
Enugu	24.7	22.4	2.3
Federal_Capital_Territory	20.4	18.8	1.7
Gombe	27.8	26.6	1.2
Imo	18.4	16.8	1.6
Jigawa	72.6	67.3	5.3
Kaduna	33.2	31.6	1.6
Kano	46.5	44.1	2.3
Katsina	84.0	81.1	2.9
Kebbi	100.5	95.2	5.4
Kogi	33.3	30.6	2.7
Kwara	22.0	20.4	1.6
Lagos	10.4	9.3	1.1
Nasarawa	43.4	39.6	3.8
Niger	43.1	40.7	2.4
Ogun	25.7	23.8	1.9
Ondo	45.6	41.0	4.6
Osun	35.3	32.8	2.5

TGO: Severe malaria burden (page 2)

Annual severe malaria hospitalisations per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co–delivered with EPI and have a 90% uptake relative to the coverage of the co–delivered intervention.

Admin–1	Hospitalisations per 1,000, no PMC (0–2.5y)	Hospitalisations per 1,000, with PMC (0–2.5y)	Hospitalisations averted per 1,000 (0–2.5y)
Oyo	30.9	28.7	2.2
Plateau	39.2	35.9	3.2
Rivers	25.4	22.7	2.6
Sokoto	103.4	99.6	3.8
Taraba	32.3	30.7	1.6
Yobe	46.1	43.2	2.9
Zamfara	102.3	99.4	2.9